

 Department of Health & Human Services		Licensing & Background Checks		Inspection Checklist		This tool is a preliminary inspection checklist designed to ensure consistency across all inspections. <i>(Revised June 2026)</i>	
Provider Name: Wasatch Behavioral Health Special Service District dba Aspire Academy		Facility ID: F22-93708				Phone Number: (801) 373-4760	
Site Address: 371 S Vineyard Rd Orem, UT 84058				Email Address: kdowns@wasatch.org; lolson@wasatch.org; khughes@wasatch.org			
Approved Capacity: 16		# of Present Residents\Clients: 9					
Please review the following items prior to the inspection: (Mark with a check mark if completed and make any necessary notes)				Please review the following items during the inspection: (Mark with a check mark if completed and make any necessary notes)			
☐	Current backgrounds in DACS	Not assessed during quarterly inspection		☒	Any active rule variances	Medication packaging	
☐	Current staff roster collected	Not assessed during quarterly inspection		☐	Introduce yourself and any DHHS staff		
☐	Any license restrictions or conditions	No conditions or restrictions in place		☒	Staff Interviews	1	
☒	Any needed rule variances	None needed		☒	Clients Interviews	2	
Inspection Information:							
- These are initial observations and do not constitute a final inspection report. The Licensor will email you this inspection checklist after the inspection is completed if requested. An official inspection report will be sent to you once this inspection has been approved by OL management.							
Inspection Type:	3rd Unannounced	Date:	7/30/2026	Time Started On-site:	11:14 A.M.	Time Ended On-site:	11:50 A.M.
Number of Items Under Review:		0	Name of Individual Informed of this Inspection:		Kent Downs		
Licensor(s) Conducting this Inspection:		Heather Guanajuato			OL Staff Observing Inspection:		

Residential Treatment, Congregate Care - Inspection Checklist

(Revised July 2026)

R = Reviewed.

UR = Under Review. Under Review indicates evidence of potential noncompliance.

NA = Not Assessed during this inspection.

New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R380-600-3(3) An applicant or a provider shall permit the office to have immediate, unrestricted access to: (a) any unaltered on and off-site program or facility and client records; (b) each client who independently consents to speak to OL staff; (c) each site subject to licensing or certification; and (d) each staff member.	☒	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☒	☐	☐	☐	☐	
R380-600-3(15) The license or certificate holder shall adhere to any individualized parameter on a program or facility license or certificate to promote the health, safety, and welfare of any client. Parameters may include: (a) adequate square footage to determine capacity; (b) an admission or placement restriction; or (c) an age restriction.	☒	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R380-600-3(25) The provider shall post their current license or certificate, except in a foster home, on the premises in a place readily visible and accessible to the public.	☒	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. (b) The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. (b)(i) The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. (ii) The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: (i) OBP approves a subsequent application; or (ii) the denial is overturned in an administrative hearing or by the OBP director. (b) The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Variances	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-6(5) The provider shall sign the approved variance and comply with the terms of the written variance, including any conditions or modifications contained within the approved written variance.	☒	☐	☐	☐	☐	
Inspection and Investigation Process	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-7(16) When a critical incident occurs under the direct responsibility and supervision of the program or facility, the provider shall: (a) submit a report of the critical incident to OL in a format required by OL within one business day of the critical incident occurrence; (b) additionally ensure any allegation of an incident of abuse, neglect, or exploitation of a client is reported to DCFS for a minor client or APS for an adult client and law enforcement within 24 hours; (c) notify the parent or legal guardian of each involved client within a 24-hour period from the time of the incident; (d) if the critical incident involves any client in the custody of the department or under contract with the department, notify the involved department division immediately; and (e) collect, maintain, and submit original witness statements and supporting documentation, including video footage if available, regarding each critical incident to OL upon request.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Scope/License Required	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-3. Scope. Each provider shall comply with any applicable federal, state, or local law, rule, or ordinance, including: (1) Section 26B-2-123; (2) Section 26B-2-124; (3) Rule R380-80; (4) Rule R380-600; (5) Rule R500-4; (6) Rule R501-14; and (7) this rule.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-4. License Required. (1) A license is required for any provider offering residential treatment, congregate care program services to any client: (a) under the age of 12 with OL approval; (b) ages 12 through 17; and (c) ages 18 through 19 only as outlined in this rule. (2) A residential treatment, congregate care license is not required for: (a) a youth shelter; (b) a youth receiving center; or (c) any other short-term or temporary setting for a youth.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-19A-5(1) Policies, Procedures, and Safe Practices. (1) Each provider shall develop, implement, and comply with policies, procedures, and safe practices for: (a) behavior management; (b) gender non-discrimination; (c) identifying and responding to a child in crisis; and (d) suicide prevention.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-5(2) The provider shall ensure staff receive training on the policies, procedures, and safe practices described in this section.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-5(3) The provider shall ensure compliance with the policies, procedures, and safe practices described in this section by: (a) submitting the required policies, procedures, and safe practices to OL during initial and renewal licensure and when any policy change requires approval; and (b) making any policy, procedure, or safe practice available upon request to OL and a client's parent or legal guardian.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-5(4) The provider shall ensure any policy, procedure, and safe practice is trauma-informed.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-5(5) The provider shall provide staff guidance regarding: (a) the appropriate response to any inappropriate behavior from other staff or a client; (b) the appropriate use of restraint, any emergency safety intervention, and seclusion; (c) prohibition of chemical and mechanical restraint unless approved by OL; (d) when a client poses a risk of violence; (e) what constitutes contraband and how the provider ensures restriction of any contraband and dangerous weapon or material; (f) appropriate client body cavity and strip search techniques, in alignment with Subsection 26B-2-123(1)(a), including that any body cavity or strip search: (i) cannot be a standard practice; and (ii) shall only be conducted by a licensed medical professional outside the view of any direct care staff; (g) critical incident reporting policy, in alignment with Rule R380-600 and Subsection 26B-2-124(1)(c); (h) any emergency procedure during any natural disaster, extreme weather event, fire, utility or structural failure, or other unexpected disruption, including: (i) continuity of care; (ii) critical incident reporting; and (iii) transport, relocation, and client health and safety; (i) how to recognize and report abuse and neglect; (j) how to report and respond to significant criminal activity or any significant medical emergency; (k) the least restrictive staff response to a client leaving the program without permission; (l) how to file a staff grievance and how to direct a client grievance; (m) the management of any unique provider circumstance regarding client supervision, the physical facility, community safety, and comingling populations; (n) firearm policy; (o) food service policy, including: (i) how to identify and accommodate any client with special dietary needs; (ii) if the program restricts access to food and kitchen equipment, an allowance for nutritious snacks to be available during restricted hours; and (iii) if any client participates in meal preparation, a policy that addresses: (A) any menu planning and procedure; (B) any nutrition requirement; (C) any sanitation requirement; (D) required training for safe food handling practices with a requirement for documented justification of each client's participation; and (E) any program rule or privilege involving kitchen use; (p) how to prevent and control any infectious and communicable disease, including coordination with and following the guidance of state or local health authorities, Center for Disease Control and Prevention, and the Department of Health and Human Services (department); and (q) provider and client responsibility for medication, including how to: (i) administer medication, including any self-administration process; (ii) log each medication dose; (iii) record and report any medication error; (iv) monitor and record any intended effect and side effect of medication; (v) record each medication dosage according to any prescription; (vi) store and administer medication on-site; (vii) store and administer medication when a client is in an off-site program-related activity; (viii) properly dispose of any unused or expired medication; and (ix) ensure the individual responsible for the discharge of a client is given any unused medication.	☒	☐	☐	☐	☐	
R501-19A-5(6) The provider shall train staff and ensure the use of any alternative sleeping arrangement, outside the client's assigned bedroom, complies with the suicide prevention plan, as described in Subsection 26B-2-124(4)(a)(iii). (a) Any alternative sleeping arrangement shall: (i) preserve client dignity and confidentiality; and (ii) be done on an individualized, time delimited basis. (b) Any alternative sleeping arrangement may not be used: (i) as a behavior management technique or as a consequence for behavior; (ii) to address any staffing shortage; or (iii) for staff convenience.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-5(7) The provider shall preserve the rights of each client who receives congregate care residential treatment services.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-5(8) The provider shall ensure there is a policy addressing management of client funds that requires documentation of each deposit and expense.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-5(9) The provider shall ensure there is a policy addressing the care, licensure, maintenance, and vaccination of any onsite animal in accordance with any local ordinance, including the: (a) assessment of any pet allergy for any client interacting with any animal in the program; (b) maintenance of any required examination, registration, and vaccination of each animal; and (c) supervision of any client in the presence of an animal.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-6(1) (a) Each provider shall provide information through print or website sources, on any program service for any client to: (i) OL; (ii) the public; (iii) any potential client; and (iv) a client's parent or legal guardian. (b) The information described in Subsection (1)(a) shall include: (i) a description of each service; (ii) each program requirement and expectation; (iii) current and accurate program contact information; (iv) the complaint reporting and resolution process; (v) identification of each non-clinical, extracurricular, or supplemental service offered or referred; and (vi) each cost, fee, and expense for a service and refund policy.	☐	☐	☐	☐	☒	Not assessed during quarterly inspection
R501-19A-6(2) The provider shall maintain a phone number to be used for the purposes described in Subsections 26B-2-124(4)(g) through 26B-2-124(4)(h).	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(3) The provider shall make every reasonable effort to connect any authorized contact who calls the program in an attempt to contact a client with that client by phone.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-1-19A-6(4) In conspicuous places where each visitor, staff, and client may see, the provider shall post: (a) abuse reporting laws, as described in Sections 26B-6-205 and 80-2-609; (b) an Americans with Disabilities Act notice; (c) any department notice of agency action; (d) a civil rights notice; (e) a client rights poster, except in a setting serving a client population governed by the settings final rule; (f) a congregate care ombudsman notice form, in compliance with each requirement in Subsection 26B-2-124(7)(a); (g) a provider code of conduct poster; and (h) the program's license.	☒	☐	☐	☐	☐	

R501-19A-6(5) In addition to posting the congregate care ombudsman notice described in Subsection (4)(f), the provider shall ensure each client, the client's parent or legal guardian, OL, and each sending agency or private agency receives a copy of the congregate care ombudsman notice.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(6) The provider shall: (a) document notification to the local government for any new service or increased consumer capacity, as described in Section 26B-2-117; (b) maintain compliance with, or documentation of any exemption from, requirements for: (i) a food handler permit for any person preparing meals for any other person; (ii) each capacity determination, including each staff and client on the premises, that does not exceed any capacity limit placed by a local authority; (iii) fire clearance, if conducted separately from a business license; and (iv) licensure and registration of any vehicle used to transport a client.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(7) Any provider with local requirements under dispute must resolve the dispute before OL may issue a license in good standing.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(8) (8)(a) The provider shall maintain: (i) fire insurance; (ii) general liability insurance; (iii) professional liability insurance, as applicable; (iv) vehicle insurance and registration for any vehicle that transports a client; and (v) any additional insurance as required to cover each program activity. (b) The provider shall make documentation of insurance and registration described in Subsection (8)(a) available to OL upon request.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(9) The provider shall ensure: (a) a manager is immediately available when the program is in operation and there is a qualified designee when the manager is absent or unavailable; (b) the implementation of a quality improvement plan addressing any: (i) client or staff grievance; (ii) program feedback; or (iii) trend in licensing noncompliance and incident reports; (c) there is a means to communicate with each client, which may include a language interpreter being available or a referral to an appropriate resource; (d) availability of a current staff list and client list; (e) each entity associated with the provider understands and signs the provider code of conduct before working with any client; and (f) the organizational and governance structure of the program is available to OL upon request and includes: (i) each line of authority and responsibility; and (ii) a job description for each job title, including each duty and qualification.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(10) The provider shall notify OL of any program change, as described in Section R380-600-4.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(11) Any provider serving or likely to serve any client with a substance use disorder shall ensure: (i) there is an opioid overdose reversal kit on site; and (ii) staff receive training on how to use the kit before being on-duty.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-6(12) Any provider serving a client who is classified as an education entitled child shall: (a) comply, as applicable, with Section 26B-2-116, Rule R277-709, and Rule R277-926 regarding coordination of any educational service, including completion of youth education forms at initial and renewal licensure; and (b) make any necessary accommodation to allow a client to continue that client's education with a curriculum approved by the Utah State Board of Education.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(13) Any provider that offers on-site school shall: (a) ensure each client is taught at the appropriate grade level; (b) ensure a staff-to-client ratio of one staff to every four clients in the school setting, unless the provider is an intermediate secure treatment program; (c) ensure staff providing school receive training in behavior intervention. (d) keep the education record of each client; (e) use a curriculum with an educational accreditation in good standing, including the Utah State Board of Education or the National School Accreditation Board; and (f) during discharge, provide proof of high school graduation or equivalent to the client, the client's parent or legal guardian, and the local education agency (LEA).	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(14) The provider shall ensure any: (a) clinical and medical staff have the appropriate license or certification; (b) clinical and medical staff are in good standing with any license or certification; and (c) unlicensed staff are appropriately supervised in compliance with Title 58, Occupations and Professions.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(15) Any provider using telehealth for treatment shall operate within the scope of the provider's professional licensure in accordance with: (a) Section 26B-4-704; and (d) any applicable rule.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(16) Any provider offering any behavior intervention service to a client served by the Division of Services for People with Disabilities (DSPD) shall comply with Rule R539-4, which supersedes any conflicting rule under Title R501.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(17) (a) The provider shall ensure access to a medical clinic or a licensed medical professional familiar with the program and population served. (b) A licensed medical professional shall oversee the medication management practices for the program. (c) The provider shall ensure that each person involved with the prescription, administration, or dispensing of any controlled substance maintains the appropriate medical or pharmaceutical license and the Drug Enforcement Administration registration numbers, as described in 21 CFR 1301 (2021), published by the Office of the Federal Register, incorporated by reference in this rule. (d) The provider shall ensure at least one CPR and first aid-certified staff is available when any staff or client is present, unless a licensed medical professional is present on site.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(18) Any provider that serves a client who has been placed in a Utah program from outside of Utah shall comply with Title 80, Chapter 2, Part 9, Interstate Compact on Placement of Children.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-6(19) The provider may allow a client to remain in the program after the client turns 18 years old, as described in Subsection 26B-2-104(1)(a)(iii), if: (a) the client has remained in the custody of a state entity or the client was admitted and continuously resided in the program for at least 30 days before the client's 18th birthday; (b) the client has a documented need to remain in the program; (c) the provider maintains responsibility for discharge to an appropriate setting when clinically appropriate and no later than the client's 19th birthday; (d) the provider outlines a plan for the protection of any client younger than 18 years old by supervising and separating the client who is 18 years old from any client who is under the age of 16; and (e) the client signs a consent document outlining that: (i) any criminal offense committed may result in being charged as an adult; (ii) if the client is involved in any critical incident posing a risk to the health and safety of any other client, the client who is 18 years old may be discharged from the program; and (iii) the client consents to remain in the program voluntarily and understands the client is not being required to remain in the program against the client's will.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(20) The provider shall ensure weekly, confidential communication between the client and any family member or authorized contact of the client, in accordance with Section 26B-2-123. (a) A client's parent or legal guardian shall authorize, in writing, an alternate means of confidential communication when voice-to-voice communication is unavailable. (b) The frequency or form of the confidential communication requirement may only be modified if the provider submits a voice-to-voice variance notification to OL, signed by the provider's clinical director, demonstrating: (i) any extenuating circumstances that exist outside the client treatment plan that inhibit offering voice-to-voice communication; (ii) an alternative that satisfies the requirement of weekly, confidential two-way communication; (iii) the program primarily operates in an area of limited or unreliable phone accessibility or coverage; or (iv) there is a significant risk of harm or danger to the client's emotional, mental, or physical safety by providing the client with unsupervised phone access. (c) The provider shall offer confidential voice-to-voice communication as soon as it is safe to do so. (d) A standardized statement or practice applied to every treatment plan may not be used to satisfy the requirement of individualized documentation.	☒	☐	☐	☐	☐	
R501-19A-6(21) A provider may offer any client step-down privileges, including authorized departures from the program and unsupervised time, if the provider: (a) documents in the client record and communicates to each client's direct care staff: (i) the individualized justification for the step-down privileges; and (ii) which privileges are authorized by a licensed clinical professional; (b) maintains a staff-to-client ratio of one direct care staff for every four clients present in the step-down setting; (c) obtains written parental or legal guardian consent before allowing step-down privileges; and (d) distributes a policy to each client, client's parent or legal guardian, and staff that includes: (i) a description of how each step-down privilege may be achieved or rescinded; (ii) a description of what constitutes authorized departure and unsupervised time; (iii) a statement that no client with step-down privileges is allowed to perform any direct care staff duty; and (iv) a statement that the provider will immediately communicate to each client, client's parent or legal guardian, and direct care staff when any step-down privilege has been rescinded.	☐	☐	☒	☐	☐	Licensee does not provide step-down privileges.
R501-19A-6(22) Any provider that serves any client with a substance use disorder shall ensure each staff and client is screened for tuberculosis as recommended by the local health authority.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-6(23) The provider shall make any program record in this section available to OL for review upon request.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Client Intake and Discharge	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-19A-7(1) Each provider shall develop, maintain, and adhere to an admissions review process that: (a) is approved by OL during initial and renewal licensure; (b) aligns with the admissions criteria described in Section 26B-2-124; (c) outlines the process for identifying a qualified candidate who can be safely served in the program, including an assessment of each candidate in consideration of: (i) the candidate's behavior; (ii) the candidate's diagnosis; (iii) the candidate's individual situation; (iv) the candidate's trauma history; (v) the population and age of each client the provider already serves; (vi) the physical facility; (vii) the programming; and (viii) the ability of current staff to manage the candidate; and (d) outlines how to identify a person not considered a qualified candidate.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(2) The provider may not solicit or accept payment from, or on behalf of, a client, unless: (a) the client meets the approved admissions criteria; and (b) the client's parent or legal guardian has signed a contract for the provider's services. (3) The provider shall develop, maintain, and adhere to: (a) an outline of each behavior or presenting issue that would be a reason for: (i) declining to classify a candidate as qualified for admission; or (ii) discharging a current client from the program; and (b) a statement, included in the provider's admissions marketing material, that the program does not accept placement of a client whose needs exceed the scope or ability of the program.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(3) The provider shall develop, maintain, and adhere to: (a) an outline of each behavior or presenting issue that would be a reason for: (i) declining to classify a candidate as qualified for admission; or (ii) discharging a current client from the program; and (b) a statement, included in the provider's admissions marketing material, that the program does not accept placement of a client whose needs exceed the scope or ability of the program.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(4) Before accepting any client into a program, the provider shall complete an intake screening for each client that includes: (a) a description of any presenting needs; (b) a suicide risk screening; (c) gender identity and individualized assessment for any bedroom and bathroom assignment; (d) verification that the client meets the admission criteria of the program; and (e) verification that the client does not meet any of the exclusionary criteria described in Subsection (3)(a).	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(5) A provider serving any client with a substance use disorder may not admit a client who is unresponsive or unable to consent to care because the individual is experiencing convulsions or delirium tremens, in shock, in a coma, or unconscious.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(6) A provider serving any court-mandated, justice-involved client shall: (a) conduct a criminogenic risk assessment during the client intake process; and (b) physically separate high criminogenic risk populations from low criminogenic risk populations, including in any bedroom area.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-7(7) The provider shall ensure that the client, or the client's parent or legal guardian, signs and receives a copy of agreements to be maintained as client records, including the: (a) determination of eligibility; (b) fee agreement outlining the cost of services, including program and client, or the client's parent or legal guardian's, responsibility for payment; and (c) signed consent for treatment that outlines: (i) the rights of the client; (ii) any program expectation of a client and the client's parent or legal guardian; (iii) licensing contact information; (iv) insurance information and identification of any other entity that is billed for the client's services; (v) the client's Medicaid number, if applicable; and (vi) services to be provided.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(8) The provider shall ensure, upon admission, a disruption plan is tailored to each client and includes: (a) any plausible reason identified in the admissions process for possible discharge or transfer; (b) an aftercare plan for disruption, which includes any resource available to the client; (c) a plan for safe transportation, including: (i) any return to the client's state of origin; (ii) to the client's parent or legal guardian; (iii) to another licensed congregate care program; or (iv) to a higher level of care program or setting, as needed, that: (A) provides increased clinical treatment and enhanced supervision to address any client need; and (B) excludes any receiving center, detention facility, and any immediate crisis stabilization setting, such as an emergency room or a short-term acute care hospital; (d) a signed statement from the client's parent or legal guardian outlining the plan for the client in the event of an unplanned disruption in care; (e) current emergency contact information of the client's parent or legal guardian, including the parent or legal guardian's: (i) name; (ii) address; (iii) phone number; and (iv) email address; (f) each individual responsible for the client's return if placement at the facility disrupts; and (g) a statement acknowledging the program retains jurisdiction and responsibility for the client while the client remains in Utah.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(9) The provider may not serve any client from out of state without a disruption plan, as described in Section 26B-2-124.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(10) A provider serving any client from out of state shall comply with Section 80-2-905, as applicable.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(11) The provider may demonstrate compliance with Subsection (9) by producing the ICPC-100A and ICPC-100B forms as required by the ICPC Regulations or a disruption plan.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(12) Any provider that transports an out-of-state client to a health care facility, as defined in Section 78B-3-403, shall comply with the payment requirements described in Subsection 26B-2-124(10).	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
(R501-19A-7(13)) Before the provider may accept or discharge any client who is transported by a youth transportation company, the provider must ensure the youth transport company is registered with OL.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-7(14) The provider shall ensure the completion and documentation of an intake, following an approved admissions review process, no later than seven days after the admission date.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-7(15) Each client's intake assessment shall include: (a) any developmental, educational, psychological, social, and vocational factor; (b) a statement of how the client is a qualified candidate for admission; (c) authorization for the provider to obtain emergency care for the client; (d) the client's cultural background; (e) the client's health and medical history; (f) the client's dominant language and mode of communication; (g) the client's family history; and (h) a suicide prevention plan that is: (i) tailored to the client and; (ii) maintained and revised as needed to support the client's safety.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Client Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-8(1) The provider shall ensure each client's treatment plan is: (a) developed and signed by a licensed clinical professional within 30 days of admission; and (b) followed by staff.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-8(2) (2) Any staff working directly with a client shall be informed of the individual treatment needs of that client.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Client Records	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-9(1) The provider shall maintain a process for the retention of any client record for seven years or until a client turns 21 years old, whichever comes later.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-1(A-9(2)) The provider shall make any client record available to OL upon request.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-9(3) The provider shall ensure each client record includes: (a) the client name, date of birth, and identified gender; (b) accurate parent or legal guardian contact information; (c) a list of any client's authorized contact as identified by the client's parent, legal guardian, or sending agency or private agency, including each authorized contact's: (i) name; (ii) phone number; and (iii) relationship to the client; (d) any signed agreement and consent form; (e)(i) consent for each treatment and non-clinical service signed by the client, or the client's parent or legal guardian; or (ii) a court order of commitment to services in lieu of signed consent for each treatment and non-clinical service; (f) any information that could affect the health, safety, or well-being of the client, including any: (i) allergy; (ii) chronic condition; (iii) communicable disease; and (iv) medication needs; (g) any grievance or complaint made by or against the client and any action taken by a provider; (h) each crisis intervention or critical incident report involving the client; (i) an individualized assessment for restriction of access to any on-site item that could be used as a weapon, for self-directed violence, or as an intoxicant; (j) an individualized disruption plan; (k) an individualized suicide prevention plan, as needed; (l) an intake screening and assessment; (m) any progress note and service provided with date and signature of staff completing each entry; (n) a summary of attendance and absence in each treatment service; (o) the treatment or service plan; (p) any referral arrangement made by the provider; (q) discharge documentation; and (r) an initial and annual client tuberculosis screening result for any client with a history of substance abuse.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-9(4) The provider shall disclose how any client and client's parent or legal guardian may access any educational or medical record following the client's discharge from the program, including how and where to locate records if the program is no longer licensed.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Staff Records and Training	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-10(1) The provider shall maintain staff information, including: (a) a provider code of conduct with the signature of each staff, contracted employee, or volunteer; (b) any approved and current OBP background clearance, except as excluded in Rule R501-14; (c) any applicable staff certification, license, or qualification; (d) any grievance or complaint made by or against the staff and action taken by the provider; (e) any pre-service and annual training record, which includes the: (i) date of completion; (ii) topic; and (iii) signature of each staff who completed the training; (f) each crisis intervention or critical incident report involving staff; and (g) documentation of the initial and annual tuberculosis screening, if serving a population with a substance use disorder.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-10(2) The provider shall ensure staff training is: (a) completed within 30 days of hire and annually thereafter; and (b) covers: (i) CPR and first aid; (ii) Rule R380-80; and (iii) any policies, procedures, and safe practice, as described in Section R501-19A-5.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-10(3) The provider shall ensure staff behavior management training includes: (a) client rights; (b) the provider code of conduct; (c) how staff shall address any injury or complaint; (d) de-escalation techniques and least restrictive methods for managing behavior; (e) the physiological and psychological impact of restraints or emergency safety intervention; (f) any threshold for restraints or emergency safety intervention; (g) the time limit for restraints or emergency safety intervention; (h) appropriate monitoring of any use of restraints and emergency safety intervention; (i) how to recognize the physical signs of distress or positional asphyxia and obtain medical assistance; (j) the procedure: (i) to intervene if another staff fails to follow procedure for restraints or emergency safety intervention; (ii) for documenting and reporting any use of restraints or emergency safety intervention; (iii) for processing restraints or emergency safety intervention with a client; and (iv) for staff follow-up after use of restraints or emergency safety intervention; and (k) the process for obtaining clinical approval for continued use of restraints or emergency safety intervention.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Physical Facility	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-11(1) (a) The provider shall maintain the facility in a clean and safe manner free from any hazard that could pose a risk to a client's health or safety. (b) The provider shall keep each furnishing and finish clean and in good repair. (c) The provider shall conduct a fire drill at least quarterly and document the response time and process for each fire drill. (d) The provider shall ensure each appliance and electrical, HVAC, and plumbing system is maintained in operating order.	☒	☐	☐	☐	☐	
R501-19A-11(2) The provider shall provide indoor space for informal client activities.	☒	☐	☐	☐	☐	
R501-19A-11(3) The provider shall ensure that when a client is present, there is a phone available in each facility that can be used to call 911.	☒	☐	☐	☐	☐	
R501-19A-11(4) The provider shall maintain a first aid kit at each facility.	☒	☐	☐	☐	☐	
R501-19A-11(5) The provider shall have a separate space for any client who is showing symptoms of an infectious disease.	☒	☐	☐	☐	☐	
R501-19A-11(6) The provider shall ensure there is designated space available for: (i) records; (ii) administrative work; and (ii) confidential client phone calls.	☒	☐	☐	☐	☐	

R501-19A-11(7) For each client bedroom, the provider shall ensure: (a) there is at least 60 square feet per client; (b) there is a source of natural light; (c) the room is ventilated by mechanical means or equipped with a window that opens; (d) any bedroom assignment is made in accordance with OL policy and individualized assessment, described in Section 26B-2-109; (e) no client is locked in the client's bedroom; (f) each client has a similar solid type of bed or sleeping equipment to any other client in the program; (g) each client is allowed to decorate and personalize the client's bedroom, while maintaining respect for each other resident and the property; and (h) each bedroom is comparable to other similarly used bedrooms with similar access, finishes, furnishings, location, and space.	☒	☐	☐	☐	☐	
R501-19A-11(8) A provider serving any client receiving DSPD services may not house more than two clients in each bedroom.	☒	☐	☐	☐	☐	
R501-19A-11(9) The provider shall ensure any bedding or towel provided to a client is laundered: (a) weekly; (b) any time the bedding or towel is soiled; and (c) after the discharge of the client.	☒	☐	☐	☐	☐	
R501-19A-11(10) The provider shall supply each client with any hygiene item necessary to meet the client's needs.	☒	☐	☐	☐	☐	
R501-19A-11(11) The provider shall ensure each client bathroom: (a) allows for individual privacy; (b) affords reasonable accommodation based on the client's gender identity; (c) is placed to allow each client access without disturbing any other client during sleeping hours; (d) is properly equipped with toilet paper, soap, and a non-shared means to dry hands; (e) is ventilated by mechanical means or equipped with a window that opens; and (f) includes a mirror, or safety mirror, secured to the bathroom wall at a convenient height.	☒	☐	☐	☐	☐	
R501-19A-11(12) The provider shall ensure any live-in staff has a dedicated bedroom and bathroom separate from any client bedroom or bathroom.	☒	☐	☐	☐	☐	
R501-19A-11(13) If the program permits a client to do laundry, the provider shall ensure: (a) any equipment and supplies for washing and drying laundry are available; and (b) separate containers for soiled and clean laundry are provided, if the program provides common laundry for bedding, clothing, or towels.	☒	☐	☐	☐	☐	
R501-19A-11(14) The provider shall maintain any on-site medication or potentially hazardous item lawfully, responsibly, and with consideration of the safety and risk level of the population served, including locked storage for each medication and hazardous chemical that is not in active use.	☒	☐	☐	☐	☐	
R501-19A-11(15) The provider shall ensure the licensed medical professional responsible for administration and supervision of prescription medication reviews information and each dosage for each client at least weekly.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-11(16) For any on-site medication, the provider shall ensure the medication: (a) is stored in the manufacturer's original packaging, including any directions and warning information; and (b) is discarded when no longer required for a client.	☒	☐	☐	☐	☐	
R501-19A-11(17) The provider may store any on-site medication, or medication for an off-site visit, outside of the manufacturer's original packaging if, at least weekly, the provider ensures: (a) a licensed medical professional oversees the filling and administration of each medication package; (b) each client's pill organizer includes: (i) the client's name; (ii) the medication's name; (iii) any directions for medication administration; (iv) dosage information; and (v) the date filled; and (c) any medication is discarded when no longer required for a client.	☒	☐	☐	☐	☐	
Licensee does utilize seclusion rooms.	☐	☐	☒	☐	☐	
R501-19A-11(19) The provider shall accommodate a client with a physical disability as needed or refer the client to comparable services.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Background Checks	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19A-12(1) OBP shall conduct a human services program background check for each program staff or volunteer, in accordance with Section 26B-2-120 and Rule R501-14.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-12(2) The provider shall designate an individual who is responsible for: (a) ensuring and documenting how each applicant is directly supervised for the entirety of that applicant's supervised employment term before receiving the eligible determination; (b) ensuring an application is submitted in DACS within 14 days of each staff becoming associated with the provider; (c) initiating, monitoring, and maintaining background checks in DACS for each individual with direct access; (d) maintaining compliance with Rule R501-14; (e) managing communications with OBP and OL; (f) monitoring DACS and taking necessary action when staff eligibility status changes; and (g) separating any staff in DACS within five days of that staff; (i) no longer being associated with the provider; or (ii) having an ineligible determination status from OBP on a background check with no pending appeal.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-12(3)The provider shall ensure each direct care staff, contracted staff, volunteer, and intern has an eligible background check determination before permitting that individual to work unsupervised with a client, except as excluded in Rule R501-14.	☒	☐	☐	☐	☐	
Client Care, Safety, and Supervision	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-19A-13(1) The provider shall ensure: (a) a staff-to-client ratio of one direct care staff to every four clients is maintained, except: (i) to reduce the staff-to-client ratio during client sleeping hours to one direct care staff to every 16 clients; (ii) in an intermediate secure program, as specified in Subsection R501-19A-14(2); and (iii) as otherwise required by a department contract; (b) a ratio of one staff to one client during any transport only occurs when the provider has conducted a safety assessment that demonstrates client and staff safety is reasonably assured; (c) any direct care staff assigned to supervise a client one-on-one is not counted at the same time in the staffing ratio for any other client, except in an emergency situation; (d) at least two direct care staff are on-duty and immediately available; (e) direct care staff conduct and document line-of-sight check-ins at least every 15 minutes when the staff is outside the line of sight of any client; (f) direct care staff conduct and document sign-of-life check-ins during sleeping hours and when a client is in the medical bay; (g) continuous direct supervision of any client that meets supervision and ratio needs of the presenting population, including increasing the ratio for individual supervision as needed; (h) on-site video surveillance is only used to directly supervise a client in time-out or seclusion or as an enhancement to minimum supervision ratio requirements; (i) only direct care staff may perform direct supervision; (j) staff conduct and document physical check-ins every 15 minutes when a client is being monitored by video; (k) the staff-to-client ratio may only decrease during sleeping hours if: (i) a minimum of two direct care staff are on-duty; (ii) each client is appropriately supervised to ensure health and safety at the ratio; and (iii) each direct care staff remains awake while on-duty; (l) the staff-to-client ratio is increased as necessary to ensure the health and safety of the client population; (m) there are enough staff on-duty each shift to safely supervise the client population, including adding more staff than required by the usual staffing ratio as needed to manage behaviors, dynamics, and individual client treatment and supervision needs; (n) there is a phone accessible to each client, staff, or any other individual to place a direct call to the congregate care ombudsman: (i) at any time; (ii) without interference; (iii) with sufficient privacy to preclude another individual from hearing the conversation; and (iv) subject to the whistleblower protections in Section 26B-2-124.3; and (o) video surveillance is only used in a client bedroom if: (i) the provider monitors any camera footage or physically performs client checks in at intervals of at least every 15 minutes; (ii) there is a documented need; (iii) providing services to an individual served by DSPD, the video surveillance is compliant with Rule R539-3; and (iv) the client, or the client's parent or legal guardian, provides written permission.	☒	☐	☐	☐	☐	
R501-19A-13(2) The provider may not allow: (a) any client, non-direct care staff, or direct care staff who does not have current behavior management training, or anyone unfamiliar to the client, to use any form of restraint on a client; (b) any intervention that uses painful stimuli, unless as an individually justified and documented emergency safety measure; (c) the use of any physical work assignment or activity that inflicts pain as a behavior management technique; and (d) the use of restraints or seclusion as a convenience to staff, substitute for programming, or punishment.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-13(3) The provider may allow a client to be separated from the area of any activity or any other client, including a client's bedroom, if on-duty staff: (a) provides direct line-of-sight supervision and monitors the client; (b) does not prevent the client from leaving the area; and (c) documents the separation, including the reason for the separation and the amount of time the client was separated from an activity or another client.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-13(4) The provider may only use any: (a) body cavity search of a client in alignment with Subsection 26B-2-123(1)(a)(ii); and (b) strip search of a client in alignment with Subsection 26B-2-123(1)(a)(i).	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-13(5) For each critical incident, the provider shall: (a) document the details of the critical incident in each client and staff record; (b)(i) report to each involved client's parent or legal guardian no later than one business day after the incident; or (ii) when the critical incident involves any client meeting the definition of a child in crisis, report to the client's parent or legal guardian no later than five hours after the crisis began; and (c) report to OL no later than one business day after the incident.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-13(6) During each shift, the provider shall: (a) document and share with staff from the next shift and the administration any client illness, injury, or critical incident; and (b) maintain and make available to OL upon request each staff shift list.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
Additional Requirments for Intermediate Secure Care Programs	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-10A-14(1)(a) Each intermediate secure treatment provider shall clearly define in policy any responsibility of the manager, as described in Subsection R501-19A-6(9)(a). (b) The provider shall ensure the manager: (i) is at least 25 years old; (ii) has a bachelor's degree or equivalent training in a human service-related field; and (iii) has at least three years of management experience in a residential or secure treatment setting.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection
R501-19A-14(2) Each intermediate secure treatment provider shall maintain a staff-to-client ratio of one direct staff to every five clients.	☒	☐	☐	☐	☐	
R501-19A-14(3) Each intermediate secure treatment provider shall ensure that each direct care staff working in an intermediate secure treatment program: (a) receives training to work with a client with behavioral or mental health needs; (b) works under the supervision of a licensed clinical professional; and (c) completes 30 hours of additional training annually regarding: (i) client record and incident documentation; (ii) each rule and regulation associated with serving clients; (iii) interpersonal relationships and communication skills; (iv) maintaining staff, client, and visitor safety in a secure setting; (v) problem-solving and guidance; (vi) the special needs of families; and (vii) any universal precaution for bloodborne pathogens.	☐	☐	☐	☐	☐	Not assessed during quarterly inspection

R501-19A-14(4) Each intermediate secure treatment shall incorporate the use of any fixture and furnishing to limit client self-harm and suicide, including:
(a) non-exposed fire sprinkler heads;
(b) plexiglass or safety glass;
(c) pressure release robe hooks;
(d) recessed lighting; and
(e) sealed light fixtures.

